Section I Wrap-Up

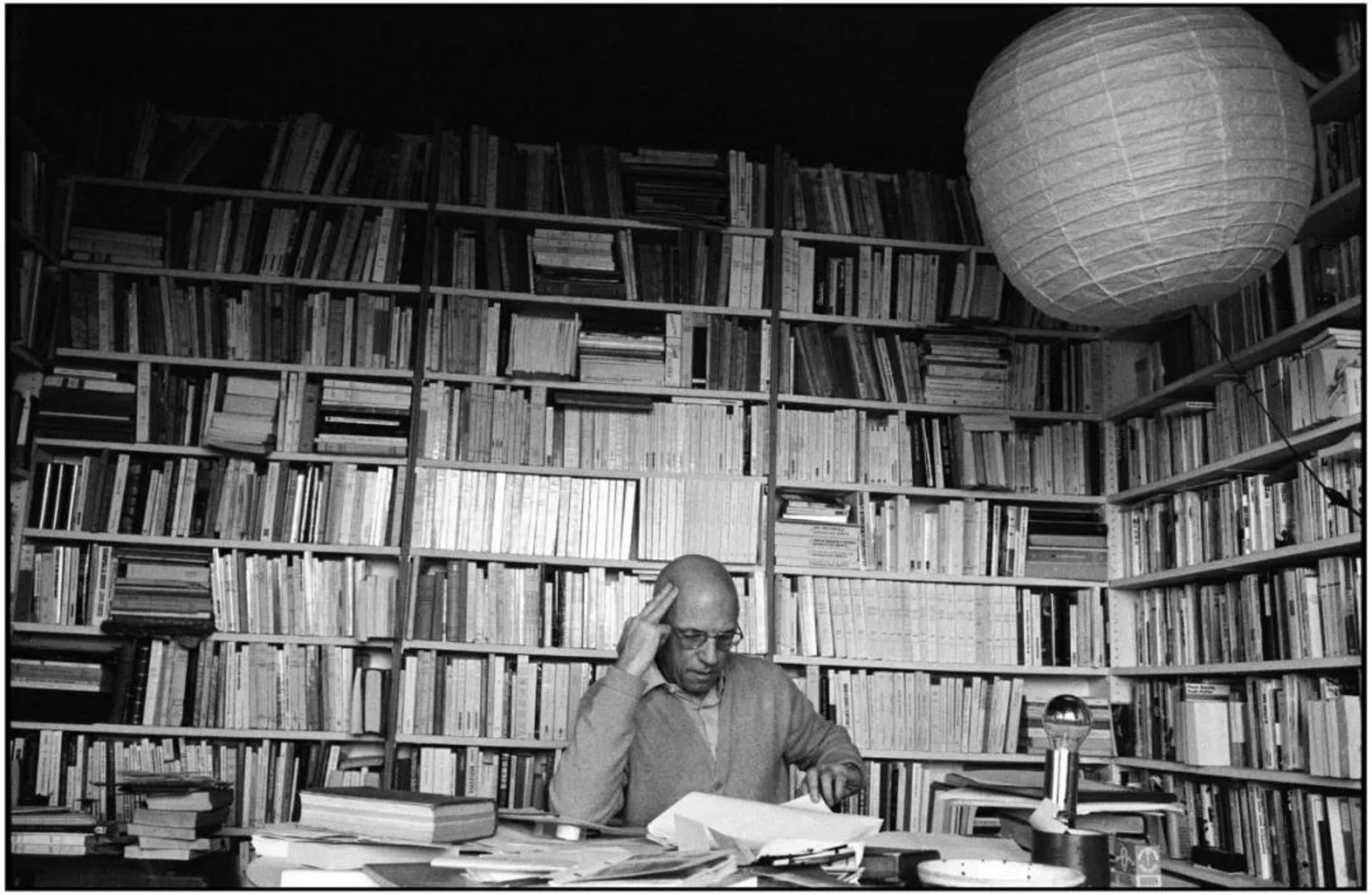
Culture and Health
Week 8, Oct 15

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Department of Sociology, Emory University



The hundredth anniversary of Michel Foucault's birth

- *"The Birth of the Clinic"*
- Biological reductionism and the **medical gaze**
  - How medical training shapes what clinicians see and pay attention to when they look at a patient
  - What becomes visible through a medical perspective
- Knowledge/power
- How medical discourse constructs knowledge about the body?



Reading Response: Grading Rubric

Criterion	Full-credit expectations	Points
Context and Central Argument	Briefly introduce the reading's context and accurately explain its main argument	2.0
Data, Case, or Reasoning	Explain how the author uses evidence, data, or cases to support the argument; for theoretical papers, explain how the author develops the argument	2.0
Textual Evidence	Include at least **two relevant direct quotations** from the reading, **with page numbers;** explain how each quotation supports your analysis	2.0
Independent Reflection	Develop your own response to the topic	1.5
Critical Assessment	Discuss potential limitations of the reading, or meaningful questions that the paper leaves unanswered	1.5
Writing and Organization	Present a clear, focused, and well-organized response of at least 700 words	1.0
Total		10.0

Reading Response: Submission Requirements

Three responses total, 10 points each, 30% of final grade

Submission Requirements

- Submit through Canvas—Assignments
- Due by 11:59 p.m. on the night before the reading is discussed
- Minimum length: 700 words
- Your Reading Presentation text **cannot** also be used for a Reading Response

In-Class Discussion

Step 1: Watch & Discuss (15 min)

- We will watch several short videos and review media coverage of this topic.

Step 2: Random Assignment (5 min)

- You will be randomly assigned to one of two positions: Defend or Critique.
- If needed, one student will serve as an observer/commentator.

Step 3: Group Preparation (15 min)

- Work with your group to develop arguments, examples, and possible rebuttals.
- Divide speaking roles among group members.

Step 4: Debate (25 min)

- Each side will present its argument, respond to the other side, and participate in open discussion.

Step 5: Debrief (15 min)

- We will reflect on how different frames shape our understandings of stigma, privacy, awareness, responsibility, and illness.

In-Class Discussion

Important:

- You are defending an assigned position, not necessarily your personal view.
- No one is expected to disclose personal health experiences.

Discussion Memo:

- Reflect on the position you were assigned, the strongest argument from the other side, and your own view after the debate.
- You may disagree with the position you were assigned.

Sep 1	**T**	“Culture, Illness, and Care: Clinical Lessons from Anthropologic and Cross-Cultural Research.”
Sep 3	Th	“Culture in Action: Symbols and Strategies.”
Sep 8	T	“Conceptualizing Stigma.” *Recommended:* Goffman, Erving. 1986. *Stigma: Notes on the Management of Spoiled Identity*.
Sep 10	Th	Conrad, Peter. 1992. “Medicalization and Social Control.” *Recommended*: Conrad, Peter. 2005. “The Shifting Engines of Medicalization.”
Sep 15	T	“The Social Construction of Illness: Key Insights and Policy Implications.” *Recommended:* “Naming and Framing: The Social Construction of Diagnosis and Illness.”
Sep 17	Th	“Morality and Health: News Media Constructions of Overweight and Eating Disorders.”
Sep 22	T	“Prescriptions and Proscriptions: Moralising Sleep Medicines.”
Sep 24	Th	“Dep®ession and Consom♀tion: Psychopharmaceuticals, Branding, and New Identity Practices.”
Sep 29	T	“The Relationship between Cultural Tightness–Looseness and COVID-19 Cases and Deaths: A Global Analysis.”
Oct 1	Th	“Culture Wars and COVID-19 Conduct: Christian Nationalism, Religiosity, and Americans’ Behavior During the Coronavirus Pandemic.”
Oct 6	T	“Economies of Dying: The Moralization of Economic Scarcity in U.S. Hospice Care.”

Sickness, Disease, and Illness

Sickness

The general condition of being unwell

Disease

- **The condition identified by medical professionals**
- Abnormalities in the structure and function of body organs and systems
- The primary focus of biomedicine: diagnosing and treating disease

Illness

- **The lived experience of being unwell**
- Personal, interpersonal, and cultural reactions to disease/discomfort
- May occur in the absence of disease

The Illness Process

Personal awareness of a change in body feeling

⇩

The **labeling as "ill"** of the sufferer

⇩

Potential action for recovery: **Seek advice** from community members, professionals, and "marginal" practitioners

Illness as a cultural experience

- Illness is culturally shaped in the sense that how we perceive, experience, and cope with disease is based on **our explanations of sickness**
- The explanations are specific to the social positions and **the systems of meaning**
- Illness behavior is thus a normative experience governed by cultural rules: we **learn "approved" ways of being ill**

The illness process may not include registration within the legitimized health system



Personal awareness of a change in body feeling

⇩

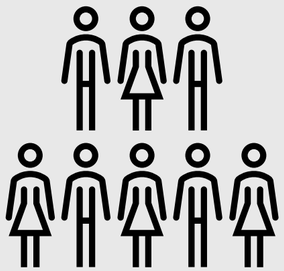
The **labeling as "ill"** of the sufferer

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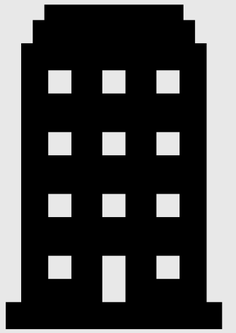
Potential action for recovery: **Seek advice** from community members, professionals, and "marginal" practitioners

Medical profession





Society



Pharmaceutical company



Toolkit/ Repertoire

Individual



Treatment

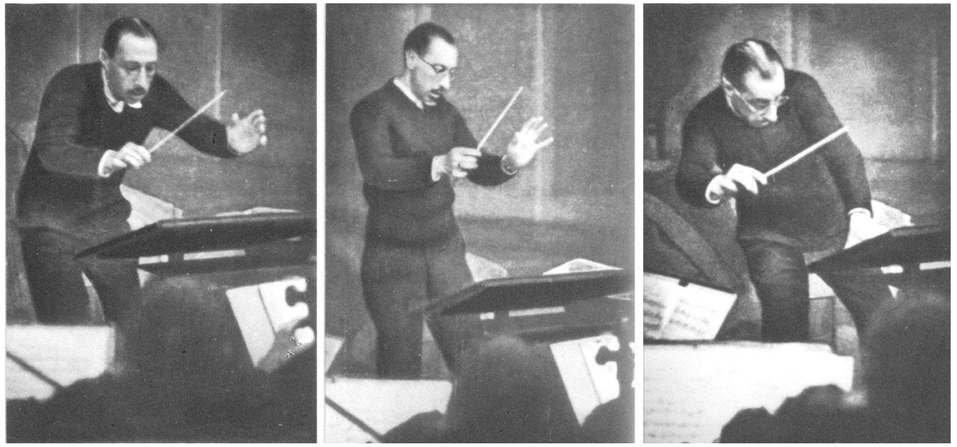
Cultural “Tool Kit”

- Culture is a **“tool kit”** of symbols, stories, rituals, and worldviews
- Culture shapes action by providing **cultural components** that are used to construct **“strategies of action”**



- Actors select differing pieces from this “tool kit” for constructing lines of action
- Culture has an independent causal role, because it **shapes the capacities** from which such strategies of action are constructed

Repertoires in the Toolkit



- The collection of works that a performer knows, is ready to perform, and selects according to the occasion, audience, and purpose
- Availability, familiarity, selection, and performance
- In sociology: **available ways of speaking, interpreting, evaluating, and acting**



"A culture is not a unified system that pushes action in a consistent direction. Rather, it is more like **a 'toolkit' or repertoire** from which actors select differing pieces for constructing lines of action."

(Swidler 1986)

Medical profession

Stigma

Society

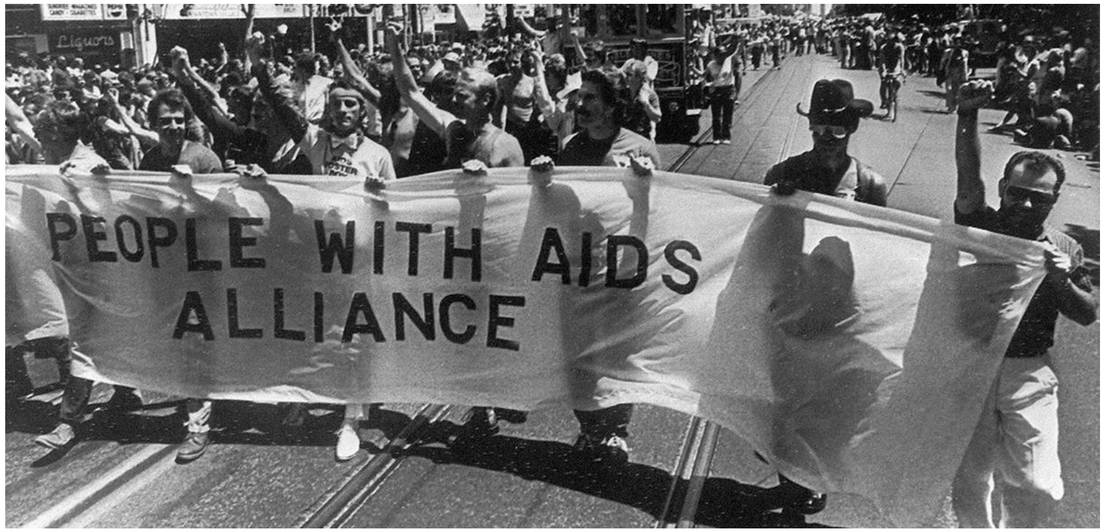
Pharmaceutical company

Toolkit/ Repertoire

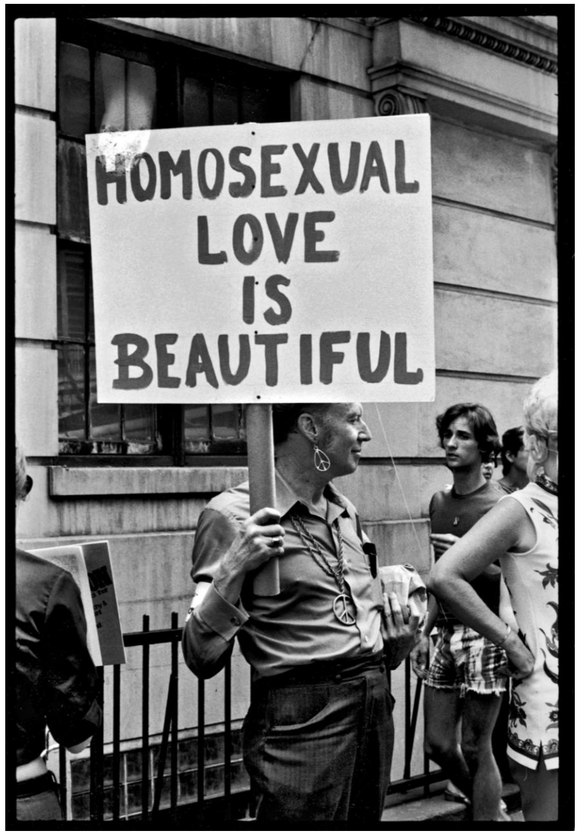
moralize

Individual

Treatment







Stigma:
Some **attributes** are related to **undesirable characteristics** derived from **stereotypes** that do not represent fact

human differences

Vast majority ⇨ Get ignored

Certain differences ⇨ **Become salient**

Come to identify differences that will matter socially;

Become "labels" affixed to the stigmatized

Social selection

⇨

Status Loss

- Reduced status in the eyes of the stigmatizer

Discrimination

Body As A Moral Object

- Thinness → discipline/self-control/status
- Fatness → gluttony/laziness/lack of control

Anorexia/Bulimia

Overweight/Obesity

Both involve eating/body size/medical risk

"No One to Blame"

"No One to Blame but Yourself (and Your Parents)"

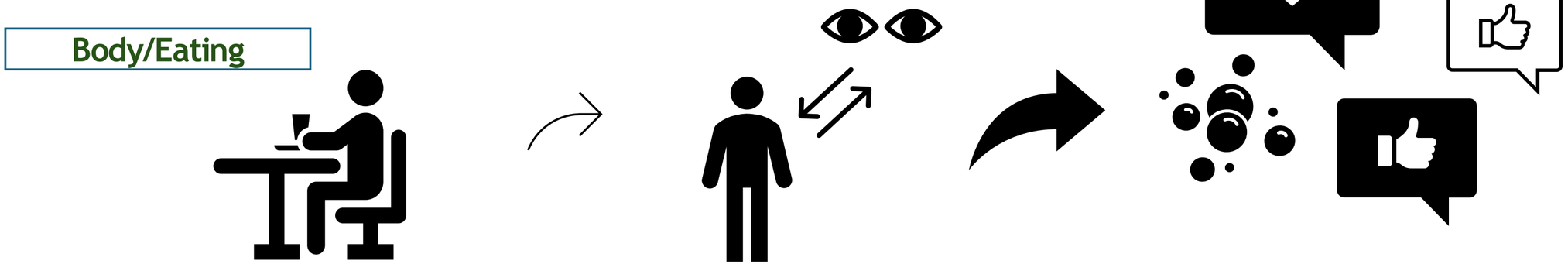
Social Status

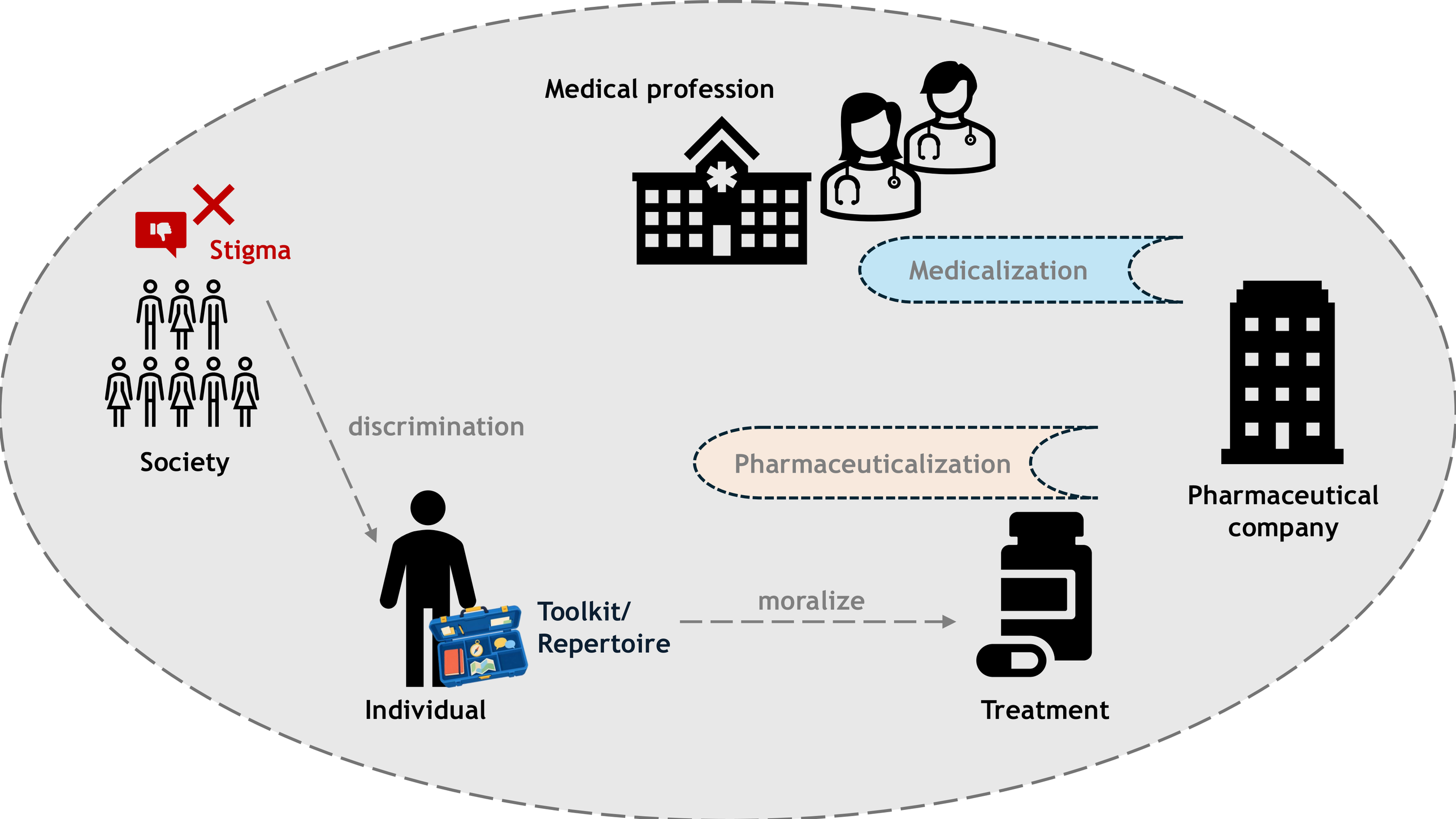
Moral Values/Virtues

Moral Meanings

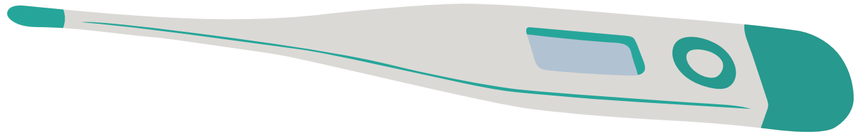
Frames/Gaze

Body/Eating




Medical profession
Stigma
Medicalization
Society
discrimination
Pharmaceuticalization
Pharmaceutical company
Toolkit/ Repertoire
moralize
Individual
Treatment

Medicalization



A sociocultural process that means "to make medical":

Medicalization occurs when a medical framework or definition is applied to understand or manage a problem

Changes since the 1980s

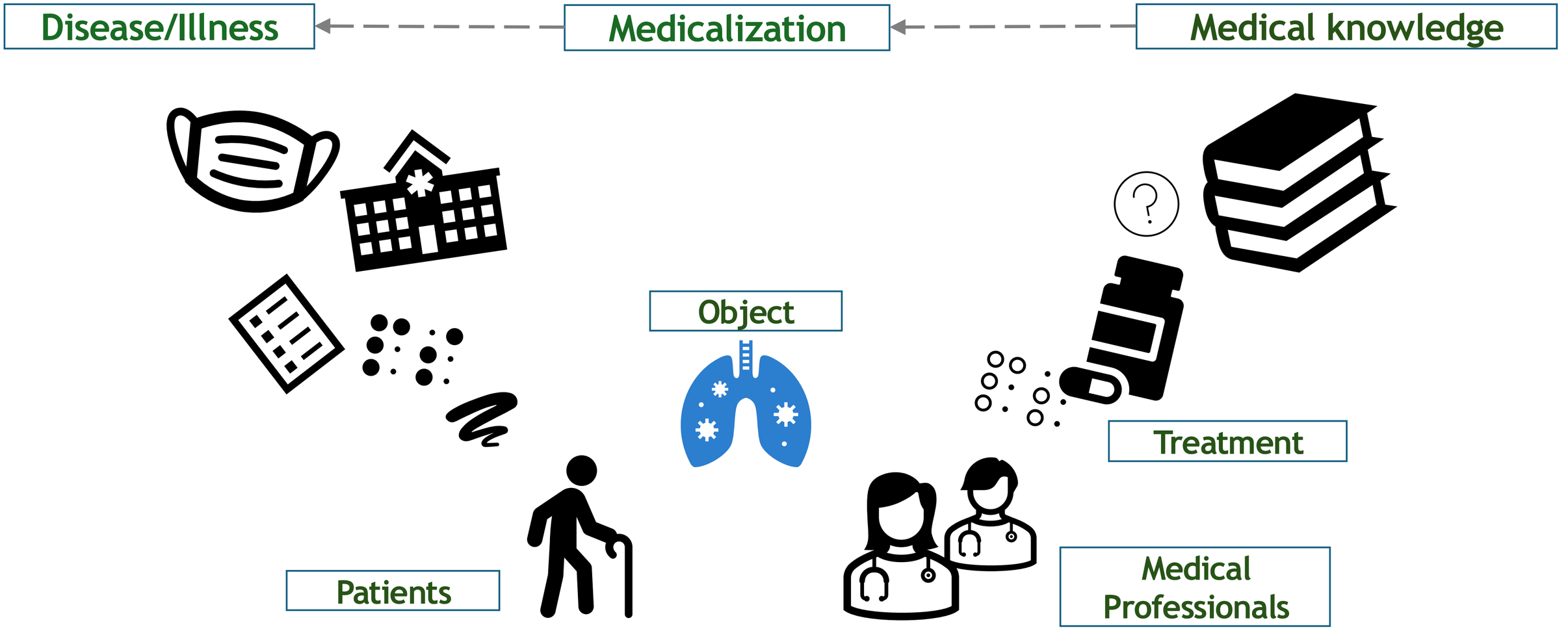
- Pharmaceutical industry
  - **Marketing via TV advertisements**, direct-to-consumer
  - "Potentially ill": constructing new status
- Consumers:
  - Increasingly vocal and active
  - Cosmetic surgery; adult ADHD; …
- Managed Care: double-edged sword
  - Arbiter for what treatment is deemed medically appropriate/inappropriate
  - May restrict treatment while simultaneously encouraging diagnosis

The social construction of illness

- Illness experience, lay experience
- Interactional approach

The social construction of medical knowledge

- The origins of professional beliefs
- Diagnosis: processes and categories

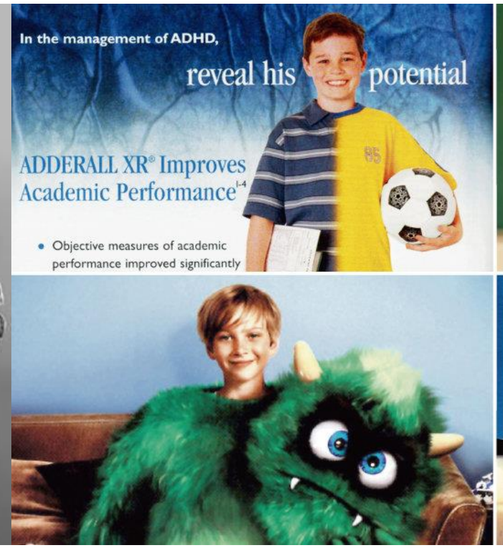


ADHD (Attention-Deficit/Hyperactivity Disorder)

Pharmaceutical companies as moral entrepreneurs

- Promote both stimulant treatment and the diagnostic category
- Advertisements describing disruptive classroom behavior as evidence of a medical disorder
- Encourage physicians to diagnose it, sponsor conferences, and fund research
- Present medication as the appropriate response and distribute diagnostic materials



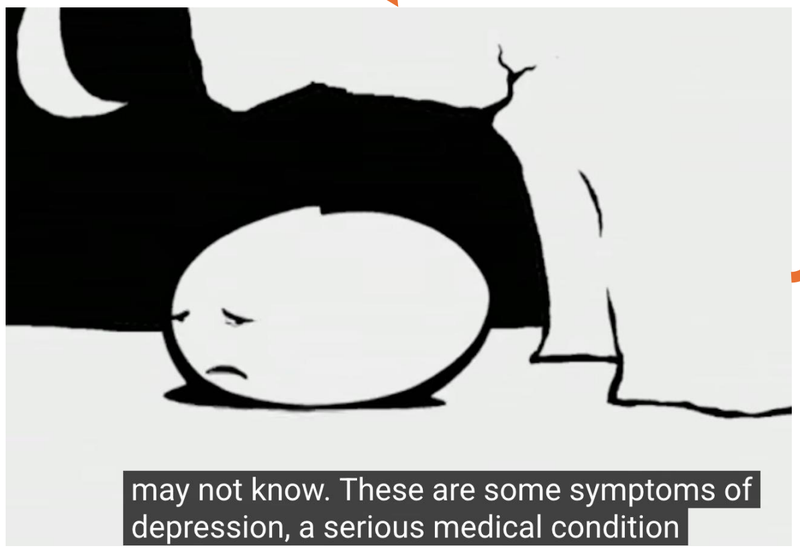




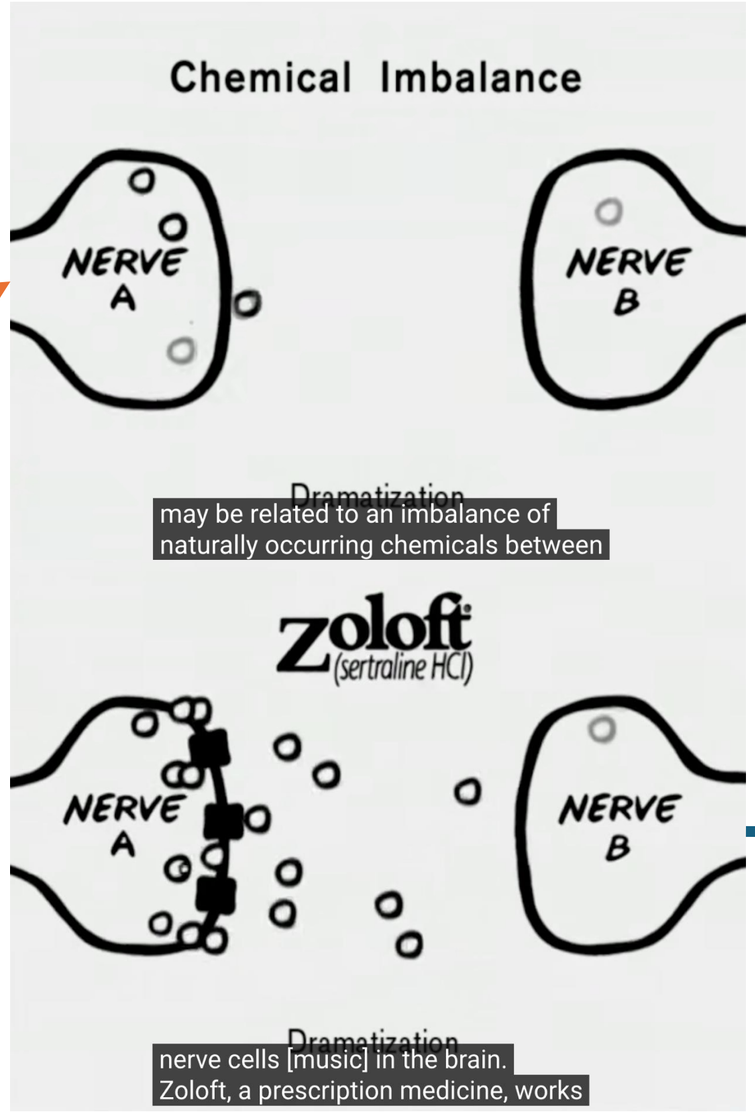




You know, when you're not feeling like yourself, you're tired all the time. You

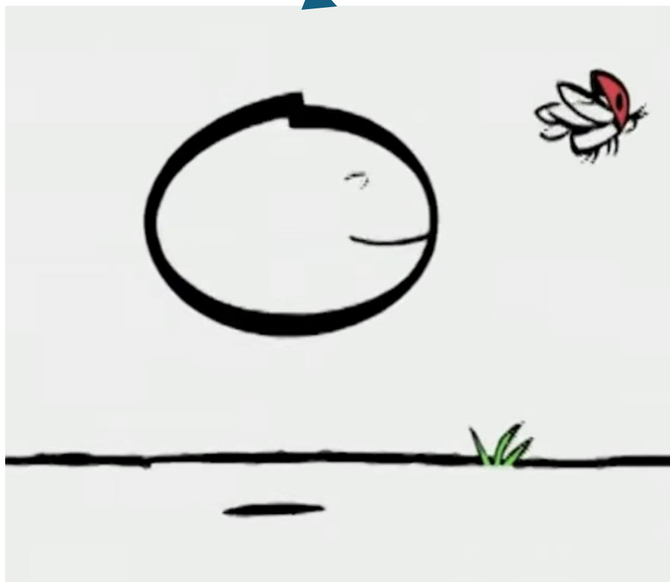


may not know. These are some symptoms of depression, a serious medical condition



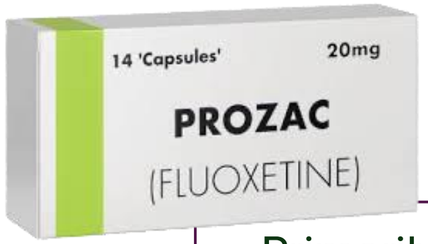
may be related to an imbalance of naturally occurring chemicals between

nerve cells [music] in the brain. Zoloft, a prescription medicine, works



Same Molecule, Different Medicine: Branding Identity

name, color, packaging,
diagnosis, patient identity, etc



Primarily associated with depression

Green-and-white capsule

A widely recognized psychiatric drug



fluoxetine

Lilly



Marketed for PMDD (Premenstrual dysphoric disorder)

Pink-and-lavender capsule

A treatment designed and branded specifically for women

Prescription drugs are simultaneously chemicals and texts

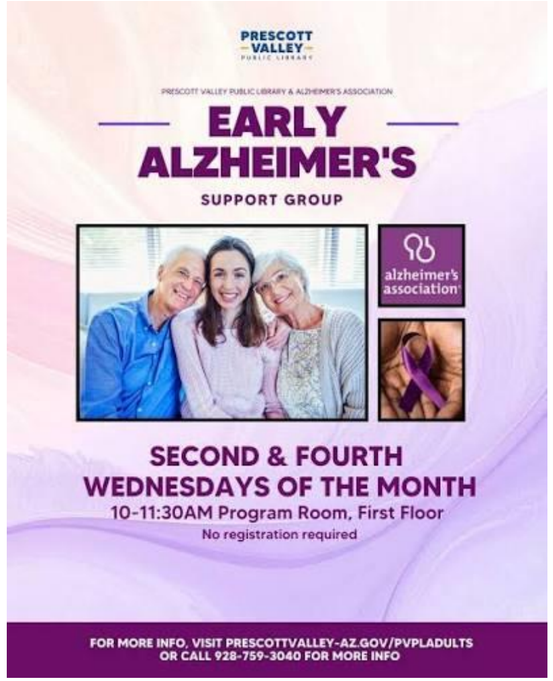
Advertisements perform versions of illness and **invite viewers to recognize themselves** within those performances

Socially Construction ≠ Unreal/Fake/Conspiracies



Social movement and interest groups:

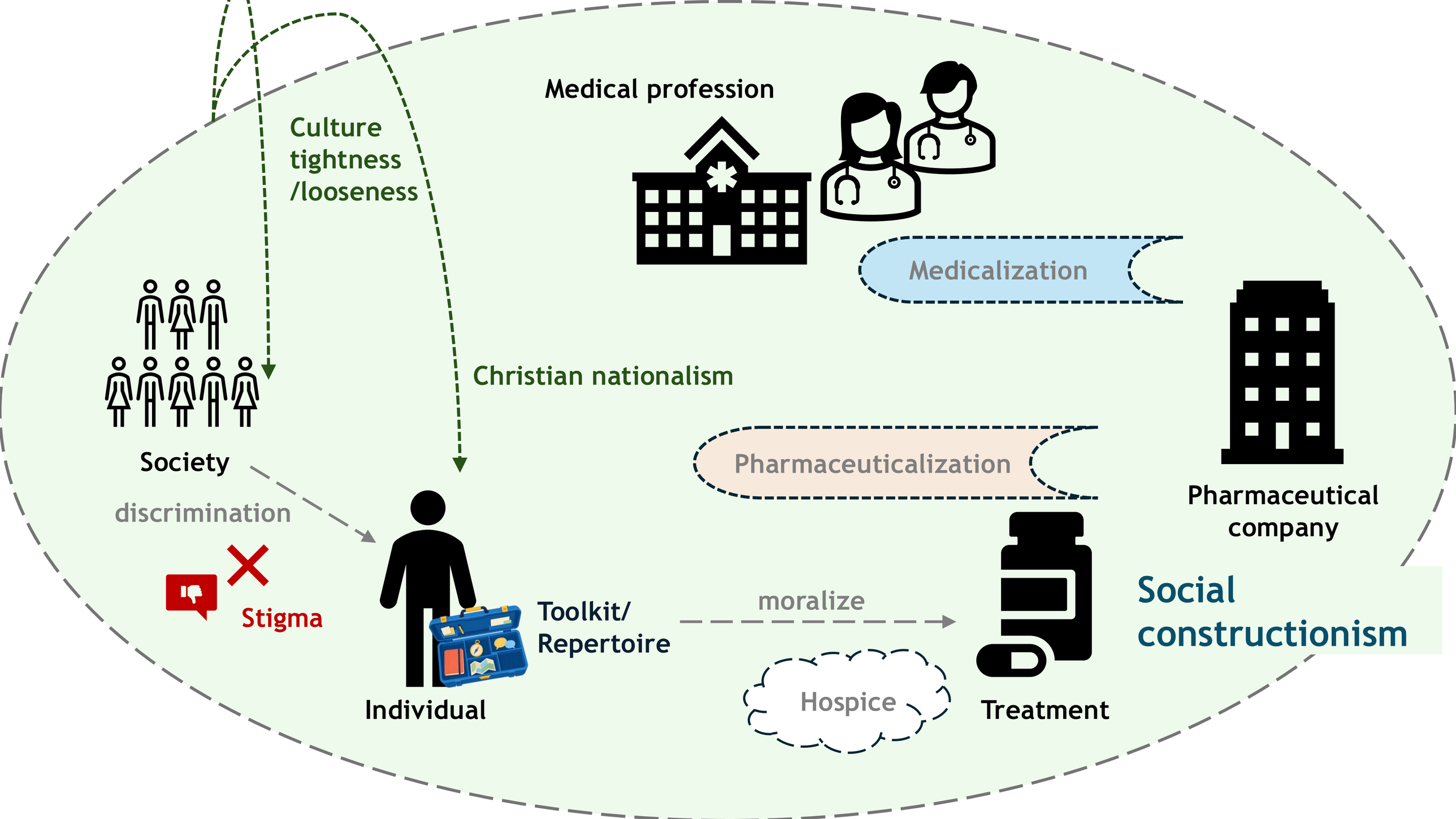
- Vietnam veterans campaigning for the medicalization of PTSD *(post-traumatic stress disorder)*
- Alzheimer's advocacy organizations



People use medical classification to validate suffering, secure resources, and establish an accepted disease identity

Hong Kong Anorexia

- The news coverage gave the public a name and a story for this condition
- A "symptom pool"
- Patients unconsciously strive for recognition and legitimization
- "Illness negotiation"


Culture
tightness
/looseness
Medical profession
Medicalization
Christian nationalism
Society
discrimination
Stigma
Pharmaceuticalization
Pharmaceutical
company
Toolkit/
Repertoire
Individual
moralize
Hospice
Treatment
Social
constructionism